

DYNA FERROUS FUMARATE TABLET

MAL19971382XZ

DESCRIPTION:

TABLET

Colour : Brown
Shape : Round, Biconvex and Scored
Coating : Film-coated

EACH TABLET CONTAINS:

Ferrous Fumarate 200 mg

PHARMACODYNAMICS:

Iron is an essential component in the physiological formation of hemoglobin, adequate amounts of which are necessary for effective erythropoiesis and the resultant oxygen transport capacity of the blood. A similar function is provided by Iron in myoglobin production. Iron also serves as a cofactor of several essential enzymes, including cytochromes that are involved in electron transport. Iron is necessary for catecholamine metabolism and the proper functioning of neurotrophils. When taken orally, in food or as a supplement, Iron passes through the mucosal cells in the ferrous state and is bound with the protein transferrin. In this form, Iron is transported in the body to bone marrow for red blood cell production.

PHARMACOKINETICS:

Iron is irregularly and incompletely absorbed from the gastrointestinal tract, the main site absorption being the duodenum and jejunum. Absorption is usually increased in conditions of Iron deficiency or when given in the fasting state. Absorption of Iron may be reduced in certain disease states. Protein binding is very high (about 90% or more).

INDICATION:

For the treatment or prophylaxis of Iron deficiency anaemia.

RECOMMENDED DOSE:

2 to 3 tablets daily, in divided doses, or as directed by the physician.

CONTRAINDICATION:

Except under special circumstances, this medication should not be used when the following medical problems exist: hemochromatosis or hemosiderosis. Risk-benefit should be considered when the following medical problems exist: Alcoholism, allergies, asthma, hepatic function impairment, kidney disease, intestinal tract inflammatory conditions, peptic ulcer, rheumatoid arthritis or sensitivity to Iron.

WARNING AND PRECAUTIONS:

Iron salts should not be given to patients receiving repeated blood transfusions or to patients with anaemias not produced by Iron deficiency unless Iron deficiency is also present. Oral Iron therapy should not be administered concomitantly with parenteral Iron. Care should be taken when given to patients with Iron storage or Iron absorption diseases, haemoglobinopathies, or existing gastrointestinal disease. The absorption of Iron salts and tetracyclines is diminished when they are taken concomitantly by mouth. If treatment with both drugs is required, the Iron salt should be administered 2 hours before or 3 hours after the tetracycline. The absorption of Iron salts may also be decreased by some antacids. Iron salts reduce the effects of penicillamine. The response to Iron may be delayed in patients receiving concomitant chloramphenicol therapy.

DRUG INTERACTIONS:

Combination containing any of the following medications (depending on the amount present) may interact with this medication: acetohydroxamic acid, alcohol, tetracyclines, etidronate, penicillamine or trientine, pancreatin or pancrelipase, dimercaprol, chelating agents, antacids and calcium or zinc supplements.

SIDE EFFECTS/ADVERSE REACTIONS:

The oral administration of Iron preparations sometimes produces gastrointestinal irritation and abdominal pain with nausea, vomiting, diarrhoea or constipation.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Iron overdose may have corrosive effects on the gastrointestinal mucosa; necrosis and perforation may occur; stricture formation may subsequently follow. Symptoms may include epigastric pain, diarrhoea, nausea, vomiting and haematemesis. Circulatory failure may follow if the diarrhoea and haemorrhage are severe. Hours or days later, after apparent recovery, metabolic acidosis, convulsions, and coma may occur. If the patient survives, symptoms of acute liver necrosis may develop and may lead to death due to hepatic coma. In treating Iron poisoning, speed is essential to block absorption of Iron from the alimentary tract. Wash out the stomach with a 1% solution of sodium bicarbonate as quick as possible and then to give 5 to 10 g of desferrioxamine mesylate in 50 to 100 ml of water to chelate any Iron left in the stomach and prevent further absorption. To eliminate Iron already absorbed, 1 to 2 g of Desferrioxamine Mesylate may be given intramuscularly or intravenously by slow infusion. If desferrioxamine is not available, empty the stomach immediately by emesis and lavage using a 1 to 5% solution of Sodium Bicarbonate, and leave up to about 300 ml of the solution in the stomach; Sodium Bicarbonate forms poorly absorbed Ferrous Carbonate with any ferrous ions. Other measures include correction of lost fluids.

PACKING/PACK SIZE(S):

Blister pack of 3x10's and 100x10's.

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

FOR ORAL USE ONLY

RECOMMENDED DAILY ALLOWANCES:

MINERALS	Unit	Infants	Children Under 4 Years of Age	Adults and Children 4 or More Years of Age	Pregnant & Lactating Women
Iron	mg	15	10	18	18

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. 198001011897 (65683-V)

2497, Mk 1, Lorong Perusahaan Baru 5,

Kawasan Perusahaan Perai 3,

13600 Perai, Pulau Pinang, Malaysia.

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Latest Revision: Nov 2022